

CLINICAL AUDIT OF CASE-MIX, DIAGNOSTIC YIELD, AND PRACTICE PATTERNS - LESSONS AND SURPRISES FROM A NEWLY ESTABLISHED BREAST SERVICE IN JOHOR

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BACKGROUND

Baseline case-mix and diagnostic data are essential to guide staffing and service planning — yet these data are rarely available in a unit's first year. We audited core needle breast biopsies from the inaugural year of the breast service at Hospital Pasir Gudang (HPG), Johor.

OBJECTIVES

Primary objective is to audit the adequacy of tissue sampling obtained in core needle breast biopsy. Secondary objective is to assess the demographic pattern of case in HPG. Expected outcomes are determining adequacy of practice and providing data for future service planning.

METHODS

Retrospective audit of all core needle breast biopsies in the first 12 months since inception of the breast diagnostic service of HPG — 91 patients, complete and anonymized. Each report was classified by diagnostic category, biopsy method (blind/palpation-guided vs. ultrasound/mammogram-guided), and tissue adequacy as determined by the full histopathology report. Receptor profile (ER, PR, HER2, Ki-67) was recorded for invasive carcinomas. Adequacy across methods was compared using Fisher's exact test.

CONCLUSION / TAKE-HOME MESSAGE

1. Main finding

91 cases audited
30.8% Malignant (n=28)
HER2-54% Positive (13/24 tested)
Adequacy - blind 80.3%, USG - 85.7%
(not significant $p=0.77$)

2. Clinical meaning

This first-year audit establishes baseline case-mix and diagnostic yield for a new breast service in HPG, Johor, spanning its transition from blind to ultrasound-guided biopsy capability.
Although blind-vs-ultrasound adequacy did not differ significantly ($p=0.77$), the resulting burden of 12 repeat biopsies carries real time-to-treatment and associated financial and temporal costs — supporting continued prioritization of ultrasound-guided biopsy now that capacity has expanded.

3. Next step

The substantial malignancy rate and notably high, possibly referral-biased, HER2-positivity further underscore the value of early-service data for anti-HER2 therapy budgeting and oncology and breast surgery service planning.
In addition, the shift to USG-guided biopsies should be accelerated to avoid repeated sampling of blind biopsies.

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